

BIOSYNC CLINICAL LABORATORY REPORT — URGENT

Patient: Jordan Riley (DOB: 04/11/2003) | Date: April 22, 2026

Vital Signs & Biometric Readings

Heart Rate	124 BPM
Blood Pressure	156/102 mmHg
SpO2	88%
Temperature	102.4°F
Glucose	103 mg/dL
Weight	122 lbs
BMI	20.3
HRV	21 ms

Clinical Notes

Patient brought in via emergency. Reports severe chest pain, shortness of breath, and inability to lie flat. Father Carlos Riley accompanying. Currently on Hydroxychloroquine 200mg twice daily, Methotrexate 15mg weekly, Prednisone 15mg daily, Sertraline 75mg, Ferrous Sulfate 325mg. Heart rate critically elevated at 124 BPM — tachycardia with palpitations reported. Blood pressure 156/102 mmHg — Stage 2 hypertension, antihypertensive initiated. SpO2 critically low at 88% — supplemental oxygen applied immediately. Pericardial friction rub heard on auscultation — lupus pericarditis confirmed on bedside echo. Temperature 102.4°F — significant fever indicating major inflammatory event. HRV 21 ms severely depressed. Patient in acute distress — IV methylprednisolone initiated. Lupus nephritis screen ordered.

Provider Notes

Lupus pericarditis confirmed — patient admitted to cardiac unit. IV methylprednisolone 1g daily pulse therapy initiated for 3 days. Cardiology managing pericardial effusion — echocardiogram showing small-moderate effusion. Nephrology consulted for lupus nephritis screening — urinalysis showing proteinuria. Methotrexate held during acute phase. Biologic therapy (Belimumab) fast-tracked for approval. Continuous cardiac monitoring. Father updated on critical status. Patient serious but stable. Full multidisciplinary team engaged. Ordering Provider: Dr. Sandra Okeke, MD — URGENT. Facility: BioSync Clinical Health Center — Emergency Dept